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CONSULT FOR AFRICA

# Dr Bola Akinola: The Referrer Engine

**How to build the pipeline that actually drives an elective specialist practice**

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## Why this is the highest-leverage work

The strategy diagnostic made one point above all: elective, high-value surgery is won on trust and referral, not on location or advertising. The evidence is blunt. When doctors refer a patient to a specialist, they decide on **reputation and relationship, not on published outcomes**. They send patients to the specialist they know, trust, and who communicates well and sends the patient back.

That is good news, because it means the pipeline is **built, not bought**. It is also the one asset a better-funded competitor cannot simply outspend, because it is made of relationships, and relationships take time and character, both of which Dr Bola has.

This playbook turns that into a system. It has three parts: **who to build relationships with, what to offer each of them, and the loop that makes referrals repeat**. The complex-case positioning is the engine's fuel: it is the single most powerful reason for another doctor to refer.

## 1. The core mechanic: be the surgeon other surgeons trust

The strongest referral source for a complex-case surgeon is **other surgeons and doctors**. This feels counter-intuitive (why would a competitor send you patients?), but it is exactly how elite specialist practices are built:

- A general orthopedist, a general surgeon, or a physician hits a case beyond their comfort: a failed replacement, an infected joint, a nasty revision. They do not want to attempt it and they do not want to lose the patient's trust by fumbling it. Referring it to a known specialist is the safe, professional move.
- When you treat that case well **and send the patient back**, you become their trusted destination for the hard ones. Over time, they also send you the straightforward ones, because you have earned the whole relationship.

This is the reciprocity loop. The complex-case reputation is the door-opener; the discipline of **returning every patient with a clear letter** is what keeps the door open. A referrer's deepest fear is losing their patient to the specialist. Remove that fear, explicitly and every time, and referrals flow.

**The one rule that matters most:** never keep a referred patient. Treat the problem you were asked to treat, write back promptly, and return them. Say it out loud, on the website and in person.

## 2. Who to build (referrer segments, in priority order)

| Segment                                                         | Why they refer                                                 | What they send                                      |
|-----------------------------------------------------------------|----------------------------------------------------------------|-----------------------------------------------------|
| 1. General & junior orthopaedic surgeons                        | Cannot or will not take on revision, infection, complex trauma | The high-value hard cases, the core of the practice |
| 2. Physicians: rheumatologists, endocrinologists, geriatricians | Manage the arthritis patients who eventually need replacement  | Primary elective hip/knee, steady volume            |
| 3. Physiotherapists & sports/rehab clinics                      | See musculoskeletal pain daily, patients ask them who to trust | Primary elective, early-stage patients              |
| 4. General practitioners & family clinics                       | First point of contact for joint pain                          | Broad, high-volume, mixed                           |



|                                                             |                                                                 |                                        |
|-------------------------------------------------------------|-----------------------------------------------------------------|----------------------------------------|
| 5. A&E and trauma units                                     | Complex fractures they cannot reconstruct                       | Complex trauma and limb reconstruction |
| 6. Diaspora networks & Nigerian medical associations abroad | Diaspora children seeking a trusted surgeon for parents at home | Diaspora-funded elective and complex   |
| 7. Existing and past patients                               | Word of mouth is the strongest of all                           | Everything                             |

Start narrow. **Segments 1 and 2 are the priority**, because they map directly onto the complex-case position and onto steady elective volume. Do those well before widening.

### 3. What to offer each referrer

The offer is not a fee or an inducement (which is both unethical and unnecessary). The offer is **professional value**: making the referring doctor's life easier and their patient better cared for. Four things, consistently delivered:

1. **A fast, honest opinion.** Same-week response. And if the patient does not need surgery, say so, to the patient and the referrer. Nothing builds trust faster than a specialist who declines to operate when it is not warranted. It signals you are safe with their patients.
2. **The patient returned, always.** A clear, prompt letter after assessment and after treatment. Their patient, back in their care. This is the fear-remover and it is non-negotiable.
3. **Access to the surgeon, not a switchboard.** A direct line (WhatsApp or phone) for referring doctors to reach him about a case. Colleagues value being able to talk to the surgeon directly.
4. **Standing where they cannot.** He does the revision, the infection, the reconstruction they cannot, to an international standard, locally. That capability is itself the offer.

### 4. The referral loop, step by step

Make referring effortless and make the experience reliably good. The loop:

1. **Easy to refer.** A dead-simple route in: a referral form on the site, a dedicated WhatsApp line, an email. A doctor should be able to refer in under a minute from their phone.
2. **Fast acknowledgement.** The referrer gets a same-day acknowledgement that the patient will be seen, and by when.
3. **Prompt assessment, honest verdict.** The patient is seen quickly and given a straight recommendation.
4. **Close the loop, every time.** Within [48 hours] of assessment, and again after treatment, the referrer receives a concise letter: what was found, what was done or advised, what happens next, patient returned to your care. This single habit, done without fail, is worth more than any marketing.
5. **A human thank-you.** A brief, genuine note of thanks for the referral. People refer again where they feel appreciated.
6. **Track it.** Record who referred whom, and what happened. This is the practice's most valuable data. (See section 6.)

### 5. The 90-day build

A focused sprint to stand the engine up. This is deliberately doable alongside a full clinical week.



### Days 1 to 30: foundation

- Write and agree the referral promise (the four offers in section 3) and put the "For Referring Doctors" page live.
- Set up the referral channels: a form, a monitored WhatsApp line, an email, all with a named person responsible for same-week response.
- Build a simple referrer list: name the top [20 to 30] doctors and clinics in segments 1 and 2 he already knows or can reach warmly.
- Design the referral letter template so closing the loop is fast and consistent.

### Days 31 to 60: outreach

- Reach the first tranche of referrers personally: a call, a coffee, a visit, or a short talk at their unit. Not a sales pitch; a professional introduction and an offer to help with their hard cases.
- Offer to give a teaching session or case discussion at two or three hospitals or clinics. Teaching is the most respected way to become the known expert. (See section 7.)
- Every referral that comes in gets the full loop, flawlessly. Early referrers must have an excellent experience; they set the reputation.

### Days 61 to 90: reinforce and measure

- Follow up warmly with everyone reached. Thank every referrer.
- Review the tracking: who is referring, what is converting, where the gaps are.
- Widen to segments 3 and 4 (physios, GPs) once 1 and 2 are warm.
- Book the next quarter's teaching sessions.

## 6. Track it: a simple referrer system

You cannot grow what you do not measure. This does not need expensive software to start; a well-kept spreadsheet or a light CRM will do.

Record, per referral:

- Referrer name, type, and hospital/clinic
- Patient (anonymised ref), date referred, date seen
- Outcome: assessed only / surgery / declined surgery
- Whether the loop was closed (letter sent), and thank-you sent
- Value of the case

Review monthly. Watch three numbers:

- **Number of active referrers** (the pipeline's real size)
- **Referrals per month, and the trend**
- **Conversion** (referred to treated), by segment

Over time this tells you exactly which relationships to deepen and where the practice actually grows, which is also the direct evidence to settle the location question with data.

## 7. Teaching and visibility (the multiplier)

The fastest way to become the surgeon others refer to is to be **visibly, generously expert** among peers. This is not marketing; it is professional reputation, and it is the most durable referral driver there is.



- **Case discussions and teaching sessions** at hospitals, clinics and physiotherapy practices. Offer to present on revision, infection, "the failed joint replacement", or "when to refer for arthroplasty". Every session is a room full of future referrers.
- **CPD and society involvement:** be present and useful in the orthopaedic and wider medical community.
- **Grand rounds and second-opinion generosity:** being the colleague who takes the call and helps with a difficult case builds more loyalty than any advertisement.
- Tie this back to the site's **Insights** pieces and any thought-leadership content, so the online authority and the in-person authority reinforce each other.

## 8. The diaspora and word-of-mouth flanks

Two additional sources that compound over time:

- **Diaspora networks.** Nigerian medical associations abroad (for example diaspora doctor groups), and diaspora community networks, are a warm route to the children-abroad who decide and pay for a parent's surgery at home. A credible UK-trained name travels well in these rooms. Worth a deliberate, if lighter, effort once the core engine is running.
- **Past patients.** The strongest referral of all is a patient who got their life back. Every well-treated patient is asked, gently, if they would be willing to share their story (for the site) and if they know others who are suffering. This is not pushy when the outcome is genuinely good.

## 9. What good looks like in twelve months

- A named, warm list of [40 to 60] active referrers, weighted to the complex-case segments.
- Every referral closed with a letter, every time, with no exceptions. This becomes the practice's signature.
- A predictable, growing monthly flow of referred patients, tracked and understood by segment.
- A regular teaching presence in the Lagos (and Abuja) orthopaedic community.
- Hard data on where patients come from, which turns "I think it is the location" into "here is exactly what drives the practice."

The engine, once built, is self-reinforcing and very hard for any competitor to copy, because it is made of trust earned one returned patient at a time.